

Concussion Myth-Buster

Ten persistent misconceptions, clinically corrected

Each of the ten statements below is commonly believed by patients, coaches, parents, and — in some cases — clinicians. Each is also demonstrably false. Each drives a specific clinical error. Share this sheet with patients and families as an education tool; use it to audit your own practice.

Myth 1 — “A patient must lose consciousness to have a concussion”

False. Loss of consciousness occurs in less than 10% of sport-related concussions. The injury is a functional disturbance of the brain, not a signature event. Screen for concussion on the basis of mechanism + symptom pattern, not LOC.

Myth 2 — “Most concussions show up on CT or MRI”

False. Concussion is defined by the absence of macroscopic structural injury on standard neuroimaging. Negative imaging is consistent with concussion, not incompatible with it. Imaging rules out more severe injury (haemorrhage, fracture); it does not rule out concussion.

Myth 3 — “Concussion symptoms resolve within 24–48 hours”

False. Median symptom duration is 10–14 days in adults and up to 4 weeks in adolescents. Approximately 10–20% of patients develop persistent symptoms lasting months. “Feeling better” within 48 hours does not mean physiological recovery is complete.

Myth 4 — “Only contact-sport athletes are at risk”

False. Falls, motor vehicle collisions, cycling, occupational injury, and assault account for the majority of concussions in the general population. Adults over 65 are at high risk; falls account for > 60% of their hospital-coded TBIs.

Myth 5 — “A clear head CT rules out concussion”

False. A clear CT rules out skull fracture, intracranial haemorrhage, and mass effect — serious injuries that require immediate intervention. It does not rule out concussion. “Nothing on the scan” is reassuring for some things; it is not a diagnosis.

Myth 6 — “Children recover more quickly than adults”

False. The reverse is true. Children and adolescents recover more slowly than adults — typical median 4 weeks versus 10–14 days. The developing brain has higher metabolic demand and a longer window of vulnerability.

Myth 7 — “Complete rest for two weeks is the standard”

False. Cocoon therapy is retired. The current evidence-based approach (Leddy 2019; Amsterdam 2023) is **relative** rest for 24–48 hours, then graded sub-symptom threshold activity. Prolonged total rest deconditions the autonomic and vestibular systems and lengthens recovery.

Myth 8 — “Helmets and mouthguards prevent concussion”

False. Helmets significantly reduce the risk of skull fracture and facial injury. Mouthguards reduce the risk of dental and orofacial injury. Neither has strong evidence for prevention of concussion itself. Rule changes and technique coaching show greater promise at the population level.

Myth 9 — “It’s safe to return to work or play once symptoms subside”

False. Symptom resolution precedes physiological recovery. Premature return to full activity increases risk of prolonged symptoms and, in the acute window, second-impact syndrome. Staged return protocols exist for a reason.

Myth 10 — “Concussion is only a cognitive problem”

False. Concussion produces cognitive, physical (vestibular, cervical, autonomic), emotional, and sleep symptoms. Ignoring any dimension produces incomplete care. Patients who “feel fine cognitively” may still have significant vestibular, cervicogenic, or autonomic dysfunction driving their real-world disability.

For clinicians. If you hear any of these myths from a patient, coach, parent, or colleague — correct it on the spot. Misinformation drives unsafe return-to-activity, under-treatment, and diagnostic delay. Naming the myth is itself a therapeutic act.